

Standard Operating Procedure (SOP)

Hospital Operations & Cashless Claims Management

For Integration with Medi Assist TPA Services

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Prepared For: Hospital Network Partners of Medi Assist Healthcare Services

1. Purpose

This SOP defines the standardized operational procedures for hospitals empaneled with Medi Assist for:

- Cashless hospitalization
- Pre-authorization processing
- Emergency admissions
- Planned admissions
- Inpatient billing
- Discharge management
- Claims submission
- Query resolution
- Fraud prevention
- Record retention

The objective is to ensure timely patient care, regulatory compliance, accurate claims adjudication, and improved patient experience.

2. Scope

This SOP applies to:

- Front Office / Admission Desk
- Insurance Desk / TPA Desk
- Billing Department
- Medical Records Department
- Nursing Department
- Treating Consultants
- Finance Department

- Quality & Compliance Team
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3. Definitions

Term	Definition
TPA	Third Party Administrator
Cashless Claim	Claim settled directly between insurer/TPA and hospital
Pre-Authorization	Approval sought before treatment costs are incurred
Enhancement	Additional approval request when costs exceed approved amount
Final Authorization	Final approval granted before discharge
Non-Medical Expenses	Expenses not payable under policy terms
Package Rate	Pre-negotiated treatment tariff

4. Roles & Responsibilities

Insurance Desk Executive

Responsible for:

- Policy verification
 - Document collection
 - Pre-auth submission
 - Enhancement requests
 - Communication with Medi Assist
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Treating Doctor

Responsible for:

- Clinical diagnosis
 - Treatment planning
 - Medical justification
 - Documentation accuracy
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Billing Executive

Responsible for:

- Package mapping
 - Bill preparation
 - Deduction management
 - Final claim submission
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Medical Records Officer

Responsible for:

- Maintaining patient records
 - Uploading discharge documents
 - Record retention
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5. Planned Admission Process

Step 1: Patient Registration

Collect:

- Government ID
- Health Insurance Card
- Employee ID (if corporate policy)
- Doctor's admission advice

Verify:

- Patient identity
 - Policy validity
 - Network eligibility
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Step 2: Policy Verification

Insurance Desk shall:

1. Verify policy through Medi Assist portal.
2. Confirm:
 - Active coverage
 - Sum insured
 - Waiting periods
 - Exclusions
 - Co-pay clauses

Step 3: Pre-Authorization Submission

Timeline:

- Within 2 hours of admission registration.

Required documents:

- Pre-auth form
- Doctor admission notes
- Investigation reports
- Insurance card copy
- KYC documents

Mandatory fields:

- Diagnosis
- Proposed treatment
- Estimated cost
- Expected length of stay

Step 4: Approval Monitoring

Insurance Desk shall:

- Track approval status every 30 minutes.
- Respond to TPA queries within 60 minutes.
- Escalate delays beyond SLA.

6. Emergency Admission Process

Definition

Any admission requiring immediate treatment where delay may endanger life or health.

Examples:

- Myocardial Infarction
- Stroke
- Polytrauma
- Severe Sepsis

- Acute Respiratory Failure
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Procedure

Immediate Care

Treatment shall not be delayed for insurance approval.

Documentation

Within 24 hours:

Submit:

- Emergency certificate
- Admission notes
- Clinical findings
- Investigation reports

Cashless Intimation

Notify Medi Assist within:

- 24 hours from admission
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7. Inpatient Monitoring

Insurance Desk shall review:

- Daily treatment progress
 - ICU transfers
 - Surgical procedures
 - Cost utilization
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Enhancement Request Process

Enhancement required if:

- Expected bill exceeds approved amount by $\geq 10\%$.

Submit:

- Updated estimate
- Progress notes
- Investigation reports
- Procedure details

Timeline:

- Minimum 24 hours before discharge whenever possible.
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8. Surgical Case Management

Prior to surgery:

Verify:

- Procedure code
- Package eligibility
- Implant coverage

Collect:

- Surgical consent
- Anaesthesia consent
- Implant stickers (if applicable)

Maintain:

- OT Notes
 - Anaesthesia Record
 - Implant Utilization Record
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9. Billing Guidelines

Bill Preparation Standards

Bills must include:

- Date-wise charges
- Room rent
- Nursing charges
- Consultation fees
- Pharmacy charges
- Diagnostics

- Procedures
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Non-Payable Items

Separate identification required for:

- Registration charges
- Food for attendants
- Toiletries
- Telephone charges
- Visitor consumables

Patient should be informed before discharge.

10. Discharge Process

Pre-Discharge Checklist

Insurance Desk shall verify:

- Final bill completed
 - Discharge summary prepared
 - Final authorization requested
 - Query responses submitted
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Final Authorization Request

Documents required:

1. Final bill
2. Break-up bill
3. Discharge summary
4. Investigation reports
5. OT notes (if surgery)
6. Implant invoice
7. Pharmacy statement

Timeline:

At least 4 hours before planned discharge.

11. Discharge Summary Requirements

Must include:

Patient Information

- Name
- UHID
- Age/Gender

Clinical Details

- Admission diagnosis
- Final diagnosis
- Procedures performed
- Treatment provided

Outcome

- Condition at discharge
- Medication advice
- Follow-up instructions

Authentication

- Consultant signature
 - Hospital seal
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12. Claims Documentation Checklist

Mandatory Documents

Medical

- Admission note
- Progress sheets
- Nursing records
- Investigation reports
- Procedure records

Financial

- Final bill

- Receipt
- Pharmacy invoices
- Implant invoices

Administrative

- Insurance card copy
 - ID proof
 - Claim forms
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13. Query Management

TPA Query Response SLA

Query Type	Response Time
Clinical Query	4 Hours
Billing Query	4 Hours
Document Query	2 Hours
Discharge Query	1 Hour

Escalation required beyond SLA.

14. Fraud, Waste & Abuse Controls

Hospital staff shall identify:

Fraud Indicators

- Identity mismatch
- Fabricated documents
- Inflated billing
- Unnecessary admissions
- Unbundling of procedures

Reporting

All suspected fraud must be reported to:

- Hospital Compliance Officer
- Medi Assist Fraud Control Unit

Within 24 hours.

15. Data Privacy & Security

Hospital shall comply with:

- Digital Personal Data Protection Act (India)
- IRDAI Guidelines
- Hospital Information Security Policies

Patient information shall:

- Be accessed only by authorized personnel
 - Be encrypted during transmission
 - Not be shared through personal email or messaging applications
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16. Record Retention

Document Type Retention Period

Medical Records 10 Years
Claims Records 10 Years
Billing Records 8 Years
Audit Records 10 Years

17. Audit Requirements

Hospital shall permit:

- Clinical audits
- Billing audits
- Documentation audits
- Fraud investigations

Audit observations must be closed within 30 days.

18. Key Performance Indicators (KPIs)

KPI	Target
Pre-auth Submission	≤ 2 Hours

KPI	Target
Query Response	≤ 4 Hours
Final Authorization Submission	≥ 4 Hours before discharge
Claim Rejection Rate	$< 2\%$
Documentation Completeness	$> 98\%$
Fraud Incidents	0

19. Escalation Matrix

Level 1

Insurance Desk Executive

Level 2

Hospital Insurance Manager

Level 3

Hospital Operations Head

Level 4

Medi Assist Relationship Manager

Level 5

Medi Assist Network Operations Team

Appendix A – Pre-Authorization Form Data Fields

- Patient Name
- Policy Number
- Employee ID
- Hospital Name
- UHID
- Admission Date
- Diagnosis
- ICD Code
- Proposed Treatment
- Estimated Cost

- Expected Stay
 - Treating Doctor Details
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Appendix B – Cashless Claim Workflow

